

Bredicon® 75mcg Tablet

(Desogestrel 75 mcg)

COMPOSITION:

Each tablet contains: Desogestrel 75 microgram

DESCRIPTION:

White colored tablet, Round Flat Bevel Edge, one side contains logo “B/75” and other side plain.

PHARMACODYNAMICS:

This medicine is a type of hormonal contraceptive commonly known as the “mini pill” or progestogen-only pill (POP). It contains the active ingredient desogestrel, which is a synthetic progestogen, similar to the natural progestogen produced by the body.

Desogestrel is thought to act as a contraceptive primarily by preventing the release of an egg from the ovary (Ovulation). It also acts by increasing the thickness of the natural mucus at the neck of the womb, making it more difficult for sperm to cross from the vagina into the womb. By preventing sperm entering the womb, successful fertilization of any eggs that are released is less likely.

Desogestrel also acts to change the quality of the womb lining (endometrium). This prevents the successful implantation of any fertilized eggs onto the wall of the womb, thereby preventing pregnancy.

The progesterone only-pill should be taken continuously every day, unlike combined pills, which are usually taken every day for three weeks, followed by a pill-free week. This progesterone-only pill should be taken continuously at the same time every day for it to be effective at preventing pregnancy.

Mechanism of action:

Desogestrel is a progestogen structurally related to levonorgestrel that has been shown to reliably inhibit ovulation. After oral administration, it undergoes oxidative transformation in the intestinal mucosa and liver to form its active metabolite, 3-keto-desogestrel.

PHARMACOKINETICS:

Absorption: After oral dosing of Bredicon, Desogestrel is rapidly absorbed and converted into its biologically active metabolite etonogestrel (ENG). Under steady-state conditions, peak serum levels are reached 1.8 hrs after tablet intake and the absolute bioavailability of ENG is approximately 70%.

Distribution: ENG is 95.5-99% bound to serum proteins, predominantly to albumin and to a lesser extent to sex hormone-binding globulin (SHBG).

Metabolism: Desogestrel is metabolized via hydroxylation and dehydrogenation to the active metabolite ENG. ENG is metabolized via sulfate and glucuronide conjugation.

Elimination: ENG is eliminated with a mean $t_{1/2}$ of approximately 30 hrs, with no difference between single and multiple dosing. Steady state levels in plasma are reached after 4-5 days. The serum clearance after IV administration of ENG is approximately 10 L/hr. Excretion of ENG and its metabolites either as free steroid or as conjugates, is with urine and feces (ratio 1.5:1). In lactating women, ENG is excreted in breast milk with a milk/serum ratio of 0.37-0.55. Based on these data and an estimated milk intake of 150 mL/kg/day, ENG 0.01-0.05 mcg/kg body weight/day may be ingested by the infant.

INDICATION:

Contraception.

RECOMMENDED DOSE:

HOW TO TAKE BREDICON

Tablets must be taken in the order directed on the package every day at about the same time with some liquid as needed. One tablet is to be taken daily for 28 consecutive days. Each subsequent pack is started immediately after finishing the previous pack.

HOW TO START BREDICON

- *No preceding hormonal contraceptive use [in the past month].*

Tablet-taking has to start on day 1 of the woman's natural cycle (day 1 is the first day of her menstrual bleeding). Starting on days 2-5 is allowed, but during the first cycle a barrier method is recommended for the first 7 days of tablet-taking.

- *Changing from a combined hormonal contraceptive (combined oral contraceptive (COC), vaginal ring, or transdermal patch).*

The woman should start with Bredicon preferably on the day after the last active tablet (the last tablet containing the active substances), or on the day of removal of her vaginal ring or patch. In these cases, the use of an additional contraceptive is not necessary.

The woman may also start at the latest on the day following the usual tablet-free, patch-free, ring-free, or placebo tablet interval of her previous combined hormonal contraceptive, but during the first 7 days of tablet-taking an additional barrier method is recommended.

- *Changing from a progestogen-only-method (minipill, injection, implant or from a progestogen-releasing intrauterine system [IUS]).*

The woman may switch any day from the minipill (from an implant or the IUS on the day of its removal, from an injectable when the next injection would be due); an additional contraceptive method is not necessary.

- *Following first-trimester abortion.*

After first-trimester abortion it is recommended to start immediately; an additional contraceptive method is not necessary.

- *Following delivery or second-trimester abortion.*

For breastfeeding women see Pregnancy and Lactation section.

The woman should be advised to start at day 21 to 28 after delivery or second-trimester abortion. When starting later, she should be advised to additionally use a barrier method for the first 7 days of tablet-taking. However, if intercourse has already occurred, pregnancy should be excluded before the actual start of Bredicon use or the woman has to wait for her first menstrual period.

MANAGEMENT OF MISSED TABLETS:

Contraceptive protection may be reduced if more than 36 hours have elapsed between two tablets. If the user is less than 12 hours late in taking any tablet, the missed tablet should be taken as soon as it is remembered and the next tablet should be taken at the

usual time. If she is more than 12 hours late, she should follow the same advice but also use an additional method of contraception for the next 7 days. If tablets were missed in the very first week of use and intercourse took place in the week before the tablets were missed, the possibility of a pregnancy should be considered.

ADVICE IN CASE OF GASTROINTESTINAL DISTURBANCES:

In case of severe gastro-intestinal disturbance, absorption may not be complete and additional contraceptive measures should be taken.

If vomiting occurs within 3-4 hours after tablet-taking, absorption may not be complete. In such an event, the advice concerning missed tablets, as given in 'Management of missed tablets' is applicable.

ROUTE OF ADMINISTRATION:

Oral

CONTRAINDICATIONS:

Progestogen-only contraceptives should not be used in the presence of any of the conditions listed below. Should any of the conditions appear for the first time during the use of Bredicon, the product should be stopped immediately.

- Hypersensitivity to the active substance or to any of the excipients.
- Known or suspected pregnancy.
- Active venous thromboembolic disorder.
- Presence or history of severe hepatic disease as long as liver function values have not returned to normal.
- Known or suspected sex steroid sensitive malignancies.
- Undiagnosed vaginal bleeding.

WARNING AND PRECAUTIONS:

If any of the conditions/risk factors mentioned as follows is present, the benefit of progestone use should be weighed against the possible risk for each individual woman and discussed with the woman before she decides to start with Bredicon. In the event of aggravation, exacerbation or first appearance of any of these conditions, the woman should contact with her physician. The physician should then decide on whether the use of Bredicon should be discontinued.

The risk for breast cancer increases in general with increasing age. During the use of COCs, the risk of having breast cancer diagnosed is slightly increased. The increased risk disappears gradually within 10 years after discontinuing of OC use and is not related to the duration of use, but to the age of the woman when using the COC.

The risk in users of progestogen-only contraceptives (POCs), eg Bredicon is possibly of similar magnitude as that associated with COCs. However, for POCs the evidence is less conclusive. Compared to the risk of getting breast cancer ever in life, the increased risk associated with COCs is low. The causes of breast cancer diagnosed in COC users tends to be less advanced than in those who have not used COCs. The increased risk in COC users may be due to an earlier diagnosis, biological effects of the pill or a combination of both.

Since a biological effect of progestogens on liver cancer cannot be excluded, an individual benefit/risk assessment should be made in women with liver cancer.

When acute or chronic disturbances of liver function occur, the women should be referred to a specialist for examination and advice.

If a sustained hypertension develops during the use of Bredicon or if a significant increase in blood pressure does not adequately respond to antihypertensive therapy, discontinuation with the use of Bredicon should be considered.

Epidemiological investigations have associated the use of combined COCs with an increased incidence of venous thromboembolism (VTE), deep venous thrombosis and pulmonary embolism. Although the clinical relevance of this finding for desogestrel used as a contraceptive in the absence of an estrogenic component is unknown, Bredicon should be discontinued in the event of a thrombosis. Discontinuation of Bredicon should be considered in case of long term immobilization due to surgery or illness. Women with

a history of thromboembolic disorders should be made aware of the possibility of a recurrence.

Although progestogens may have an effect on peripheral insulin resistance and glucose tolerance, there is no evidence for a need to alter the therapeutic regimen in diabetics using progestogen-only pills (POPs). However diabetic patients should be carefully observed during the first month of use.

Treatment with Bredicon leads to decreased estradiol serum levels, to a level corresponding with the early follicular phase. It is as yet unknown whether the decreased has any clinically relevant effects on bone mineral density.

The protection with traditional POPs against ectopic pregnancies is not as good as with COCs, Which has been associated with the frequent occurrence of ovulations during the use of POPs. Despite the fact that Bedicon consistently inhibits ovulation, ectopic pregnancy should be taken into account in the differential diagnosis if the woman gets amenorrhea or abdominal pain.

Chloasma may occasionally occur, especially in women with a history of chloasma gravidarum. Women with a tendency to chloasma should avoid exposure to the sun or UV radiation while taking Bredicon.

The following conditions have been reported both during pregnancy and during sex steroid use but an association with the use of progestogens has not been established: Jaundice and or pruntus related to cholestasis , gallstone formation, porphyria, systemic lupus erythemaiosis (SLE), haemolytic urenic syndrome, sydenham's chorea, herpes gestationis, otosclerosis-related hearing loss, (hereditary) angioedema.

Bredicon contains < 65 mg lactose and therefore should not be administered to patients with rare hereditary problems of galactose intolerance, Lapp-lactase deficiency, or glucose-galactose malabsorption.

Medical examination/ Consultation: Before prescription, a thorough case history should be taken and a thorough gynaecological examination is recommended to exclude pregnancy. Bleeding disturbances eg, oligomenorrhea and amenorrhea should be investigated before prescription. The interval between check-ups depends on the circumstance in each individual case. If the prescribed product may conceivably influence latent or manifest disease, the control examinations should be timed

accordingly. Despite the fact that Bredicon is taken regularly, bleeding disturbances may occur. If bleeding is very frequent and irregular, another contraceptive method should be considered. If the symptoms persist, an organic cause should be ruled out. Management of amenorrhea during treatment depends on whether or not the tablets have been taken in accordance with the instructions and may induce a pregnancy test. The treatment should be stopped if a pregnancy occurs.

Women should be advised that Bredicon does not protect against HIV (AIDS) and other sexually transmitted diseases.

Reduced Efficacy: The efficacy of POPS may be reduced in the event of missed tablets, GI disturbance or comitant medication.

Change in vaginal bleeding Pattern: During the use of a POC, vaginal bleeding may become more frequent or of longer duration in some women whereas in others, bleeding may become incidental or be totally absent. These changes are often a reason for the woman to reject the method or to be noncompliant. Acceptance of bleeding pattern can be improved by offering women who have chosen to use Bredicon careful counselling on this point. Evaluation of vaginal bleeding should be done on an ad hoc basis and may induce examination to exclude malignancy or pregnancy.

Follicular development: With all low-dose hormonal contraceptives, follicular development occurs and occasionally the follicle may continue to grow beyond the size it would attain in a normal cycle. Generally, these enlarged follicles disappear spontaneously. Often, they are asymptomatic in some cases, they are associated with mild abdominal pain. They rarely require surgical intervention.

Effects on the Ability to Drive or Operate Machinery: On the basis of the pharmacodynamic profile, Bredicon is expected to have no or negligible influence on the ability to drive and use machine.

INTERACTIONS WITH OTHER MEDICAMENTS:

Interaction between OCs and other drugs may lead to break through to breakthrough bleeding and contraceptive failure.

Hepatic Metabolism: Interaction can occur with medicinal products that induce microsomal enzymes, which can result in increased clearance of sex hormones (eg, hydantoins(eg, phenytoin), barbiturates (Phenobarbital), primidone, carbamazepine,

rifampicin and possibly also oxcarbazepine, rifabutin, felbamate, ritonavir, griseofulvin and products containing St John's Wort (Hypericum perforatum). Women on treatment with any of these drugs should temporarily use a barrier method in addition to Bredicon or choose another method of contraception. The barrier method should be used during the time of concomitant drug administration and for 28 days after their discontinuation. For women on long-term therapy with hepatic enzyme inducers, a non-hormonal method of contraception should be considered.

During treatment with medical charcoal, the absorption of the steroid in the tablet may be reduced and thereby the contraceptive efficacy. In such an event, the advice concerning missed tablets as given in Management of missed tablets under Dosage & Administration is applicable.

Hormonal contraceptives may interfere with the metabolism of other drugs. Accordingly, plasma and tissue concentrations may either increase (eg, cyclosporine) or decrease.

Note: The prescribing information of concomitant medications should be consulted to identify potential interactions.

Laboratory tests: Data obtained with COCs have shown that contraceptive steroids may influence the results of certain laboratory tests, including bio-chemical parameters of liver, thyroid, adrenal and renal function, and serum levels of (carrier) proteins eg, corticosteroid-binding globulin and lipid/lipoprotein fractions, parameters of carbohydrate metabolism and parameters of coagulation and fibrinolysis. The changes generally remain within the normal range. To what extent this also applies to progestogen-only contraceptives is not known.

Incompatibilities: Not applicable.

PREGNANCY AND LACTATION:

Pregnancy

Bredicon is not indicated during pregnancy. If pregnancy occurs during treatment with Bredicon, further intake should be stopped. Animal studies have shown that very high doses of progestogenic substances may cause masculinisation of female foetuses. Extensive epidemiological studies have revealed neither an increased risk of birth defects in children born to women who used COCs prior to pregnancy, nor a teratogenic effect

when COCs were taken inadvertently during early pregnancy. Pharmacovigilance data collected with various desogestrel-containing COCs also do not indicate an increased risk.

Lactation

Bredicon does not influence the production or the quality (protein, lactose, or fat concentrations) of breast milk. However, small amounts of etonogestrel are excreted in the breast milk. As a result, 0.01 - 0.05 microgram etonogestrel per kg body weight per day may be ingested by the child (based on an estimated milk ingestion of 150 ml/kg/day). Based on the available data desogestrel may be used during lactation. The development and growth of a nursing infant, whose mother uses Bredicon, should, however, be carefully observed.

SIDE EFFECTS:

Common: Irregular bleeding, amenorrhoea, headache, weight gain, breast pain, nausea, acne, mood changes, decreased libido.

Less common: Vaginitis, dysmenorrhoea, ovarian cysts, vomiting, alopecia, fatigue, difficulty wearing contact lenses.

Rare: Rash, urticaria, erythema nodosum.

Effects on ability to drive and use machines

Bredicon has no or negligible influence on the ability to drive and use machines.

SYMPTOMS AND TREATMENT OF OVERDOSE:

No serious deleterious effects have been reported from overdose. Other symptoms may include nausea, vomiting, and in young girls, slight vaginal bleeding. Treatment should be symptomatic.

DOSAGE FORMS AND PACKAGING:

Bredicon is presented in strips of 28 tablets packed in a ply carton.

SHELF LIFE:

2 years

STORAGE CONDITIONS:

Store at room temperature (below 30°C). Do not use after the expiry date stated on the package.

DATE OF REVISION

25th June, 2014.

® Trade Mark



Manufactured by:

Renata Limited

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